NCCP 00525 · VERSION 6

Ribociclib + endocrine therapy · metastatic

HR-positive / HER2-negative metastatic breast cancer · NCCP 00525 v6 · Document revised 4 November 2026

> **Your personal plan:** this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated.

Oral treatment with a planned rest week

How this medicine supports your treatment

Ribociclib with an aromatase inhibitor or fulvestrant aims to control advanced HR-positive/HER2-negative breast cancer. The exact endocrine partner depends on previous treatment and menopausal status.

CDK4/6 INHIBITOR

Ribociclib

Slows the cell-division checkpoint

CDK4/6 checkpoint → Checkpoint inhibited → Cell cycling slows

Ribociclib blocks CDK4 and CDK6, proteins that help cells move through the cycle of division. Endocrine therapy reduces the hormone signal that can drive HR-positive breast cancer.

Keep blood and ECG appointments. Fever, jaundice, fainting, palpitations, severe rash or new breathlessness need prompt advice.

ENDOCRINE PARTNER

AI or fulvestrant

Reduces oestrogen-driven growth

Oestrogen pathway → AI or receptor blockade → Growth signal reduced

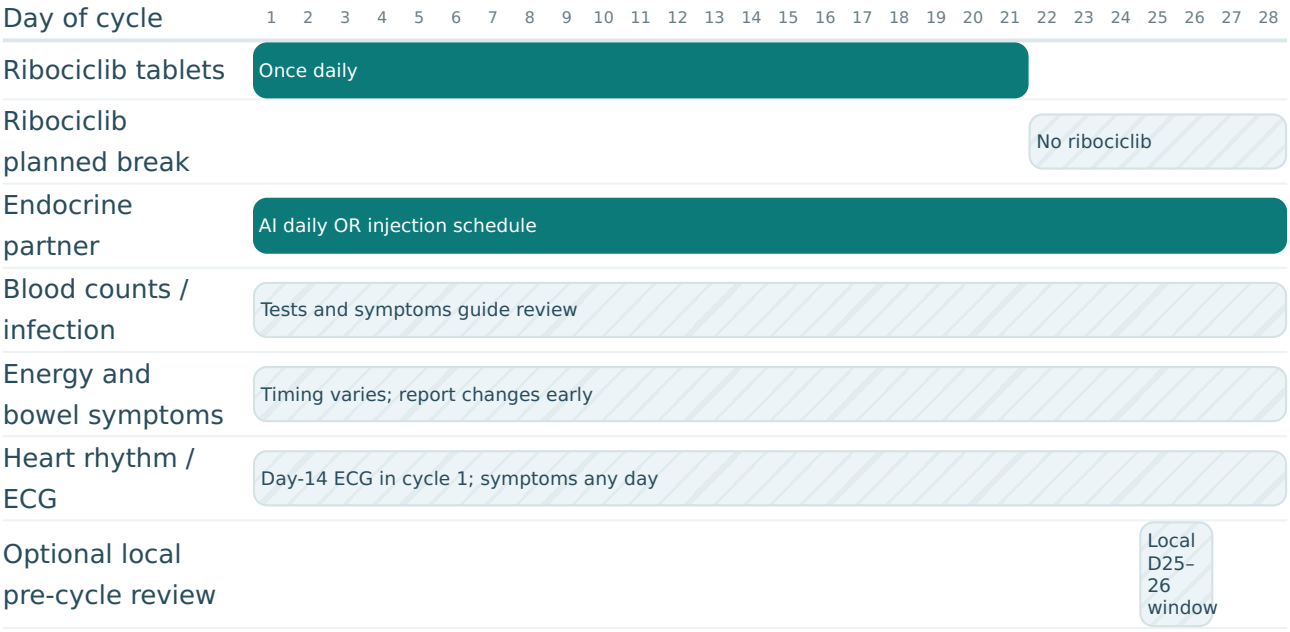
An aromatase inhibitor (AI) lowers oestrogen production; fulvestrant blocks and reduces oestrogen receptors. These are alternative endocrine partners, selected by the team. Ovarian suppression is needed in pre/perimenopausal patients.

The endocrine medicine continues during the ribociclib week off. Report troublesome joint, menopausal or injection-site symptoms.

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated.



Treatment days: Ribociclib is taken once daily on Days 1–21, followed by seven days off, in a 28-day cycle. The endocrine partner continues according to

Read the phase labels: The prescription and team's instructions determine your actual doses and any interruptions.

its own prescription. Treatment continues while it is helping and tolerable.

The bigger picture: Use the whole-treatment journey for surgery, radiation, scans and longer-term checks.

Local pre-cycle review: Day 25–26 can be a local blood / review window. It does not replace the required early checks or authorise an automatic restart.

> **Why these bands stay broad:** Treatment bands show the source schedule. Symptom bands show possible patterns, not measured probabilities; serious effects can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00525 v6 · official regimen ↗

> **There are no guaranteed "safe days".** Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override the timeline or diary.

BASELINE → TREATMENT → FOLLOW-UP

Treatment and monitoring over time

Columns show sequence and general intervals, not equal periods or booked appointments. The team confirms who arranges each test, who reviews results and who authorises treatment or restart.

Check	Before first dose	Cycles 1–2	Cycles 3–6	Later treatment	Completion / change
Ribociclib and endocrine partner	Confirm metastatic 600 mg / AI or fulvestrant plan	Days 1–21 on; Days 22–28 off; fulvestrant loading visits if chosen	Repeat if cleared; endocrine partner continues	While beneficial and tolerated	Review endocrine continuation separately
Blood counts / renal / liver	Baseline profiles	Every two weeks in both cycles	Before each cycle	As clinically indicated; more often if abnormal	Follow abnormalities before restart or after stopping
ECG / heart rhythm	Baseline ECG; check QTcF eligibility	Repeat around cycle 1 Day 14	Further ECG when clinically indicated	Repeat for symptoms, QT change or interacting medicines	No automatic restart after a QT-related hold
Electrolytes	Potassium, calcium, magnesium and phosphorus	Before each cycle; correct abnormalities	Before each cycle through cycle 6	As clinically indicated	Review unresolved abnormalities
Bone / endocrine health	Assess AI/OFS bone and menopausal risks	Support symptoms and adherence	AI/OFS and any injections follow their own plan	Repeat bone assessment according to risk	Longer endocrine and bone plan continues
Cancer response	Baseline disease assessment	Assess new symptoms promptly	Imaging according to consultant plan	Continue response and symptom review	Progression prompts new treatment decision

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

THE WHOLE TREATMENT JOURNEY

Ribociclib + endocrine therapy · metastatic: treatment and checks

HR-positive / HER2-negative metastatic breast cancer

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

On a small screen, swipe the map sideways. The checks below give the same timing in words.

Treatment, tests and reviews across the course

Care / check	Before first dose	Cycles 1–2	Cycles 3–6	Later treatment	Completion / change
Ribociclib and endocrine partner NCCP 00525 · SMPC	Confirm metastatic 600 mg / AI or fulvestrant plan	Days 1–21 on; Days 22–28 off; fulvestrant loading visits if chosen	Repeat if cleared; endocrine partner continues	While beneficial and tolerated	Review endocrine continuation separately
Blood counts / renal / liver NCCP 00525 · SMPC	Baseline profiles	Every two weeks in both cycles	Before each cycle	As clinically indicated; more often if abnormal	Follow abnormalities before restart or after stopping
ECG / heart rhythm NCCP 00525 · SMPC	Baseline ECG; check QTcF eligibility	Repeat around cycle 1 Day 14	Further ECG when clinically indicated	Repeat for symptoms, QT change or interacting medicines	No automatic restart after a QT-related hold
Electrolytes NCCP 00525 · SMPC	Potassium, calcium, magnesium and phosphorus	Before each cycle; correct abnormalities	Before each cycle through cycle 6	As clinically indicated	Review unresolved abnormalities
Bone / endocrine health NCCP 00525 · SMPC	Assess AI/OFS bone and menopausal risks	Support symptoms and adherence	AI/OFS and any injections follow their own plan	Repeat bone assessment according to risk	Longer endocrine and bone plan continues
Cancer response NCCP 00525 · SMPC	Baseline disease assessment	Assess new symptoms promptly	Imaging according to consultant plan	Continue response and symptom review	Progression prompts new treatment decision

What each check is for

1

Before the first dose

Before first dose

Check eligibility, organ function and the exact treatment/support plan.

Ask / act: Confirm who arranges each check and who reviews it.

Sources: NCCP 00525 · SMPC

2

Early toxicity checks

Cycles 1–2

Important toxicity can begin before the first routine review.

Ask / act: Keep early appointments and use the agreed symptom-contact plan.

Sources: NCCP 00525 · SMPC

3

Benefit, tolerability and adherence

Later treatment

Longer-term treatment still needs clinical review.

Ask / act: Ask about symptom support, results and the next decision.

Sources: NCCP 00525 · SMPC

4

Completion or treatment change

Completion / change

The targeted medicine and endocrine partner may have different stop dates.

Ask / act: Confirm each medicine and the ongoing follow-up plan.

Sources: NCCP 00525

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.21

Your treatment aim

Ribociclib with an aromatase inhibitor or fulvestrant aims to control advanced HR-positive/HER2-negative breast cancer. The exact endocrine partner depends on previous treatment and menopausal status.

Who this treatment is for

HR-positive/HER2-negative locally advanced or metastatic disease with an AI or fulvestrant in the exact NCCP indication. NCCP page 1 and page 2 do not fully align on menopause and previous advanced-disease treatment; preserve consultant reconciliation.

NCCP 00525 v6 · official regimen

Schedule and duration

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated. There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision.

NCCP 00525 v6 · official regimen

Checks before and during treatment

Baseline tests and ongoing tests have different purposes.

Before the first dose

FBC, renal and liver profile, potassium, calcium, magnesium and phosphorus. ECG before initiation; QTcF must be below 450 ms. Correct electrolyte abnormalities and review QT/CYP3A interactions. Confirm endocrine partner, menopause/ovarian suppression, prior treatment and the NCCP indication/eligibility discrepancy.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

During treatment

FBC, renal and liver profile every two weeks for cycles 1–2, then before cycles 3–6, then as clinically indicated. Electrolytes before each of the first six cycles, then as clinically indicated; correct abnormalities. Repeat ECG around Day 14 of cycle 1, then as clinically indicated. Increase liver monitoring for grade 2 or worse abnormalities. Review fever, jaundice, palpitations/fainting, pulmonary and severe skin symptoms; response imaging follows the advanced-disease plan.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Living with treatment

Supportive medicines and the endocrine partner have their own plan.

Nausea support

NCCP classifies ribociclib as oral moderate–high emetic risk. The team chooses preventive or rescue antiemetics using local policy, symptoms, QT risk and drug interactions; do not reuse another regimen’s prescription.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Electrolytes and heart rhythm

Correct abnormal potassium, calcium, magnesium and phosphate as directed. Attend the baseline and cycle 1 Day-14 ECG; fainting or palpitations need urgent advice.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Counts and liver tests

Keep the early two-weekly blood tests and subsequent monitoring. Report fever, shivers, jaundice or dark urine promptly.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Endocrine treatment

The AI or fulvestrant, and ovarian suppression when prescribed, continue on their own timetable. Discuss joint, menopausal, bone and injection-site symptoms.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Daily administration

Avoid grapefruit and check interactions. Do not replace a missed or vomited dose. A clinical hold requires explicit instructions about which medicine stops and who authorises restart.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

What the studies tell us

Studies of ribociclib with hormone therapy in defined advanced-breast-cancer groups showed a survival benefit. The studies used different hormone partners and included different menopausal and prior-treatment groups. Ask which study matches your plan; the results do not set an individual time limit.

How to interpret this

Study results describe groups with defined treatment and eligibility. Ask about your own likely benefits, important harms, alternatives and remaining uncertainty.

MONALEESA-2 final OS, Hortobagyi et al 2022 · MONALEESA-2 updated PFS, Hortobagyi et al 2018 · MONALEESA-3 updated overall-population OS, Slamon et al 2021 · MONALEESA-7 exploratory extended OS, Lu et al 2022

When the plan changes

There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision.

Continue, hold or stop

The team confirms the plan for each medicine and the next review. Do not restart a clinically held treatment from this educational timeline.

NCCP 00525 v6 · official regimen · EMA Kisqali current product information; checked 2026-10-25

Practical support

Nausea support

NCCP classifies ribociclib as oral moderate–high emetic risk. The team chooses preventive or rescue antiemetics using local policy, symptoms, QT risk and drug interactions; do not reuse another regimen’s prescription.

Electrolytes and heart rhythm

Correct abnormal potassium, calcium, magnesium and phosphate as directed. Attend the baseline and cycle 1 Day-14 ECG; fainting or palpitations need urgent advice.

Counts and liver tests

Keep the early two-weekly blood tests and subsequent monitoring. Report fever, shivers, jaundice or dark urine promptly.

Endocrine treatment

The AI or fulvestrant, and ovarian suppression when prescribed, continue on their own timetable. Discuss joint, menopausal, bone and injection-site symptoms.

Daily administration

Avoid grapefruit and check interactions. Do not replace a missed or vomited dose. A clinical hold requires explicit instructions about which medicine stops and who authorises restart.

Taking tablets / attending treatment

MONALEESA-2, MONALEESA-3 and MONALEESA-7: separate populations and partners

Studies describe groups. Ask how closely the treatment and study population match your situation.

Your team can discuss the strength and limitations of the evidence alongside treatment risks and alternatives.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Report new breathlessness, chest symptoms, severe diarrhoea, inability to drink, jaundice, a painful swollen leg or a severe rash promptly. You do not need to identify which medicine caused the symptom.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

Questions to ask

How long does treatment last?

The usual metastatic starting dose is ribociclib 600 mg once daily on Days 1–21 followed by Days 22–28 off. Continue the prescribed AI daily or fulvestrant on its own injection schedule; add ovarian suppression when indicated. Continue while beneficial and tolerated. There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision.

Which checks happen before and during treatment?

FBC, renal and liver profile, potassium, calcium, magnesium and phosphorus. ECG before initiation; QTcF must be below 450 ms. Correct electrolyte abnormalities and review QT/CYP3A interactions. Confirm endocrine partner, menopause/ovarian suppression, prior treatment and the NCCP indication/eligibility discrepancy. FBC, renal and liver profile every two weeks for cycles 1–2, then before cycles 3–6, then as clinically indicated. Electrolytes before each of the first six cycles, then as clinically indicated; correct abnormalities. Repeat ECG around Day 14 of cycle 1, then as clinically indicated. Increase liver monitoring for grade 2 or worse abnormalities. Review fever, jaundice, palpitations/fainting, pulmonary and severe skin symptoms; response imaging follows the advanced-disease plan.

Do I need to enter dates?

No. This map shows general timing without personal information. Your hospital arranges actual appointments; ask who books each test and who reviews its result.

What if a dose is missed or treatment is held?

Take ribociclib once daily with or without food; swallow whole. Avoid grapefruit. If a dose is missed or vomited, do not take an extra dose; take the next scheduled dose. Confirm any clinical hold and restart with the team. The endocrine partner follows a separate prescription. A hold may affect one medicine rather than the whole combination; confirm each component.

What does the evidence mean for me?

Studies of ribociclib with hormone therapy in defined advanced-breast-cancer groups showed a survival benefit. The studies used different hormone partners and included different menopausal and prior-treatment groups. Ask which study matches your plan; the results do not set an individual time limit.

What happens after treatment changes or ends?

There is no fixed three-year stop in this metastatic regimen; the team reviews response, side effects and the next treatment decision. Keep the separately agreed disease follow-up, endocrine and bone-health plan.

Explain the plan back: Which medicine is taken when, which test is due next, who reviews it, and which symptoms mean you should call today?

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00525 v6 · official regimen
- NICE NG101 · bone health and follow-up
- HSE · chemotherapy side effects and urgent advice
- ASCO · biomarkers in metastatic breast cancer
- ESMO metastatic breast cancer guideline · response assessment
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · PICC patient information
- eviQ · implanted port patient information
- Novartis · Kisqali product information, updated September 2026
- MONALEESA-2: ribociclib with letrozole · primary report
- EMA Kisqali current product information; checked 2026-10-25
- MONALEESA-2 final OS, Hortobagyi et al 2022
- MONALEESA-2 updated PFS, Hortobagyi et al 2018
- MONALEESA-3 updated overall-population OS, Slamon et al 2021
- MONALEESA-7 exploratory extended OS, Lu et al 2022

Source reconciled; independent clinical, pharmacy and patient/PPI review pending

Written source addresses

https://healthservice.hse.ie/documents/6629/525_v6_Ribociclib_Metastatic_hghzyyD.pdf

https://www.ema.europa.eu/en/documents/product-information/kisqali-epar-product-information_en.pdf

https://www.nice.org.uk/guidance/ng101/chapter/Recommendations

https://www2.hse.ie/conditions/chemotherapy/side-effects/

https://ascopubs.org/doi/10.1200/JCO.2015.61.1459

https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext

https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk

https://www.eviq.org.au/p/543

https://www.eviq.org.au/p/507

https://www.medicines.org.uk/emc/product/8110/smpc

https://pubmed.ncbi.nlm.nih.gov/35263519/

https://pubmed.ncbi.nlm.nih.gov/29718092/

https://pubmed.ncbi.nlm.nih.gov/34102253/

https://pubmed.ncbi.nlm.nih.gov/34965945/